



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض رباعياً : غاده عبدالعزيز مصطفى= في السيد

العنوان : ===

المحافظة : الشرقية

رقم البطاقة : 28210011302441 رقم التليفون : 01116854441

تاريخ تسجيل الإستمارة : 22-09-2026

العمر (تاريخ الميلاد) : 01-10-1982 المهنة : ربه منزل

الإقرار

أقر أنا الموقع أدناه أنني لا أتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات أو أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (.....) التوقيع: (.....)

المقرر بما فيه (.....) التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي علاج هرموني خط اول تاموكسيفين قسم العلاج : الاورام [14160]

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني الوظيفة : التوقيع :
الاسم : د/ندى حسن الوظيفة : التوقيع :
الاسم : د/نرمين رمضان الوظيفة : التوقيع :
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. / أ.د. / أ.د. / د. /

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى : قسم :
جهة العلاج : نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

Name: غاده عبدالعزيز مصطفي السيد
National ID: 28210011302441



تقرير طبي

السيد

تحية طيبة وبعد,,

بشأن تقرير طبي للمريض

إيماء للطلب المقدم من

رقم طبي:

بقسم:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته

تاريخ وساعة الخروج :
PM 01:17 05-07-2026

تاريخ وساعة الدخول :
-- -- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد انه

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Tamoxifen therapy

الوقت :

-- --:--

التاريخ :

05-07-2026

الطبيب المعالج :
د/شيماء لاشين

A18

w/ov

جمهورية العراق
وزارة الداخلية
مركز التسجيل والتوثيق

عبد العزيز مصطفى السيد
سجل المقيم
مركز التسجيل والتوثيق

٢٠٢٥/٠٦
٢٨٢١٠٠١١٣٠٢٤٤١

LF5581353

٢٠٢٥/٠٦
٢٨٢١٠٠١١٣٠٢٤٤١

عبد العزيز مصطفى السيد
سجل المقيم
مركز التسجيل والتوثيق

٢٠٢٥/٠٦
٢٨٢١٠٠١١٣٠٢٤٤١

Patient Name : الأستاذة / غادة عبدالعزيز مصطفى	Request No. : 13005622
Patient No. : 2821001132441	Request Date : 20/03/2024
Path No. : 1111/24	Verification Date : 27/03/2024
Sex : Female	Financial Category : نفقة الدولة - ورام دار السلام
Age : 41 Y	Department : Out Patient
Referring Doc :	Registered by : marwa mohamed youss

Pathology Report

Clinical Data: Left breast cancer, post NACT.

Nature Specimen: Left breast mastectomy & Axillary lymph nodes dissection.

Gross Examination:

- * Procedure: Left breast mastectomy measured 20x19x4 cm, with covering skin ellipse measured 17x11 cm.
- * Tumor extent: A greyish white rubbery to firm tumor bed site.
- * Size: 8 x7 x4 cm.
- * Skin: Nipple & areola looked unremarkable grossly.
- * Distance from margins: 0.3 cm away from deep margin.
- * Axilla: Fibrofatty tissue piece measured 5x5x3 cm, entangling 17 L.Ns, up to 2x1x0.5 cm

Microscopic Examination:

- * Histological type: No residual viable tumor tissue.
- Actual tumor cellularity: 0%.
- * Ductal carcinoma in situ: Not detected.
- * Skin: Free.
- * Resection Margins: Free.
- * Additional pathologic findings: Multiple fibroadenomas.
- * Lymph nodes: Free lymph nodes, L.N (0/17).
- * Histologic response to therapy: Complete pathological response to therapy in previously diagnosed invasive duct carcinoma.

Diagnosis:

- Left breast cancer post NACT, Left breast mastectomy & Axillary lymph nodes dissection:
- No residual viable tumor tissue.
 - Actual tumor cellularity: 0%.

Verified by :

Dr.Duaa Mohamed

Printing Date : 03/04/2024 11:54:14

Page 1 of 2

Printing By : marwa mohamed youss

غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Mizr El Qadema - Cairo, Egypt

Patient Name : الأستاذة / غادة عبدالعزيز مصطفى
Patient No. : 2821001132441
Path No. : 1111/24
Sex : Female
Age : 41 Y
Referring Doc :

Request No. : 13005622
Request Date : 20/03/2024
Verification Date : 27/03/2024
Financial Category : نفقة الدولة
Department : Out Patient
Registered by : marwa mohamed you

Pathology Report

- Resection Margins: Free.
- Skin: Free.
- Lymph nodes: Free axillary lymph nodes, L.N (0/17).
- Histologic response to therapy: Complete pathological response in previously diagnosed invasive duct carcinoma.
- Stage: ypT0, ypN0.

Signature
Prof. Dr. Iman Gouda.
Dr. Muhamed Turkey.

Verified by :
Dr. Duaa Mohamed

Printing Date : 03.04/2024 11:54:14

Page 2 of 2

Printing By : marwa mohamed youss



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor - Building No. 987 - Nile Corniche - Mistr El Qadema - Cairo, Egypt

IMMUNOHISTOCHEMISTRY REPORT

atient Name	الأمينة / شاد عبدالمعز مصطفى السيد	Request No.	13004450
atient No.	28210011302441	Request Date	12/10/2023
ith No.	SR-1234/23	Verification Date	02/11/2023
ex	Female	Financial Category	المرضى - نفقة الدولة
ge	41 Y	Department	Out Patient
eferring Doc.		Registered by	.marwa mohamed youssef

IMMUNOHISTOCHEMISTRY REPORT

Dr.Rana Fahty
Dr.Heba Hassan

Verified by :
Dr.Heba Hasan

Printing Date : 02/11/2023 11:46:42

Page 2 of 2

Printed By : .marwa mohamed youssef

Patient Name: Ghada Abdel Aziz Moustafa Al Sayed	Date: 20/02/2024
Exam: ultrasound BREAST (BOTH SIDE)	Age: 41 Year(s)
Medical ID: 3856/2023	Pat ID: 47061

**DIGITAL BILATERAL SOFT TISSUE MAMMOGRAPHY AND
COMPLEMENTARY ULTRASOUND**

Indication:

PNCT assessment of left breast pathologically proven IDC.

Report:

- ACR c: heterogeneous fibroglandular parenchyma
- Left Breast:
 - Irregular dense mass is seen in the UOQ with overlying skin thickening and microcalcification.
 - Ultrasound shows this mass as an irregular heterogenous lobulated mass at 12 o'clock measuring about 21x12mm, mildly reduced in size with increased echogenicity denoting partial response.
 - No residual pathologically enlarged axillary lymph nodes.
 - No supra or infraclavicular lymph node.
- Right Breast:
 - No suspicious micro calcifications or architectural distortion detected.
 - Complementary ultrasound no cystic or solid mass lesions.
 - Normal skin thickness, and nipple-areola complex with average caliber of the retro-areolar central ducts.
 - Nonspecific axillary LN.

Opinion:

- Post NACT status of left breast showing partial response of the previously noted malignant mass with no residual lymphadenopathy (BIRADS 6)
- Normal sonomammography of right breast (BIRADS 1)

Best regards,

D. Amal Ibrahim.

Dr. Amira H Radwan MD,

Breast Imaging consultant,

NCI, Cairo University

20/02/2024 3:01 PM

أ.د. خالد أحمد لكوظ
أ.د. أحمد صبري راغب
أ.د. ماجد عبدالجبار
أ.د. كريم خالد لكوظ
أ.د. أيمن فتحي أحمد
أ.د. تامر محمد إبراهيم
أ.د. محمد هشام صالح



Patient Name: محمد عبد العزيز مصطفى السيد	Study Date: 11-Sep-23
PROF DR.: محمد عبد الجواد سليمان	Patient ID: 585214

Tc-99m MDP BONE SCAN

History

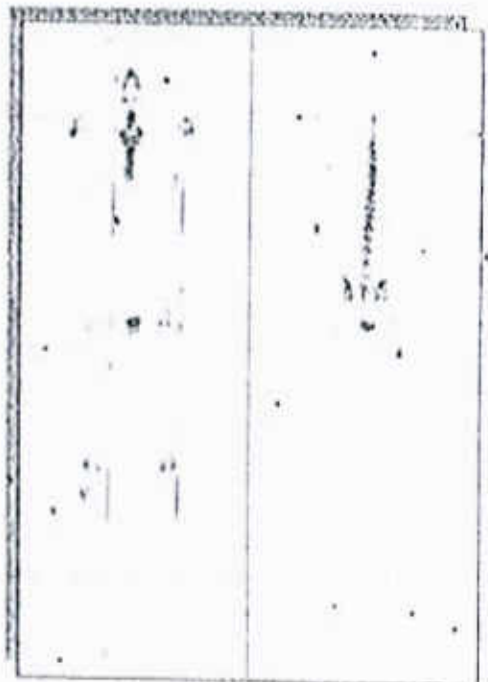
41 years old female patient presented with non-operated left cancer breast and referred for metastatic work-up.

Technique

- Anterior and posterior whole body scans were acquired 2 hours post injection of 20 mCi of the tracer.

Findings

- The current study is free from focal active osseous lesions that would account for bone metastases.
- Mild degenerative changes are noted at different levels of the spine.
- N.B. the increased tracer activity seen at the right sacroiliac region is likely of benign degenerative nature.



IMPRESSION :-

- The skeleton is free from metastases.

MUCH OBLIGED;

Dr. AMIR M. SALAH, M.Sc.

PROF. DR. MAGED ABDEL GALIL.

د. أحمد صبري راغب
د. طارق محمد إبراهيم
د. محمد هشام جمال
د. خالد لكوظ



Patient Name: د. محمد عبد الجواد مائمان	Study Date: 07/09/2023
PROF DR.: محمد عبد الجواد مائمان	Patient ID: 585214

MULTISLICE CT CHEST, ABDOMEN & PELVIS POST IV CONTRAST STUDY
REVEALS: Known case of LT. breast CA.

- Diffuse LT. breast skin thickening with dense glandular tissue and multiple LT. axillary metastatic LNs, ranging from 5-19 mm.
- Normal CT characteristics of both lungs with no focal pulmonary masses or infiltration.
- No mediastinal masses or LNs enlargement.
- No pleural or pericardial effusion.
- Average size, normal density liver with no focal masses or intrahepatic biliary dilatation.
- Normal size spleen with no focal masses.
- Both kidneys, GB & pancreas are displaying normal CT characteristics with no focal enhanced masses.
- No pelviabdominal LNs enlargement.
- Normal examined bowel loops with no focal masses.
- Normal UB.
- Normal size Uterus with an IUD in place. No adnexal masses.
- No osteolytic or sclerotic bony lesions.

CONCLUSION:

- Unremarkable CT study.

Thanks for references
Yours
Dr Tamer Ibrahim
Prof. Dr: Khaled Lakouz

أ.د. خالد أحمد لكوظ
أ.د. أحمد صبري راغب
أ.د. أيمن فتحي أحمد
أ.د. ماجي عبدالرحمن
أ.د. كريم خالد لكوظ
أ.د. تامر محمد إبراهيم
أ.د. محمد هشام صالح

Patient Name: غاده عبدالمعز مصطفى السيد	Study Date: 26/08/2023
PROF DR.: أحمد توفيق	Patient ID: 581860

**COMPUTED MAMMOGRAPHIC EXAMINATION OF BOTH BREASTS REVEAL:
CRANIO-CAUDAL & MEDIO-LATERAL, OBLIQUE VIEWS WERE TAKEN.**

-Normal parenchymal densities of both breasts with an ill-defined LT upper breast faint opaque breast mass harbors few calcifications with overlying skin thickening & nipple retraction. No other focal breast masses.

US EXAMINATION OF BOTH BREASTS REVEAL:

-Normal echogenic pattern of both breasts with LT upper breast soft tissue mass (12 O'clock) measuring about 20x18 mm with lobulated borders, showing small foci of calcifications. It is associated with overlying skin thickening & nipple retraction.
-Multiple enlarged LT axillary LNs, the largest one measures about 22x16 mm, most of them show lost central fatty hilum.
-Normal RT breast. No focal masses.

Conclusion:

- **LT upper breast mass as described ...(BIRADS-IVc), For biopsy if further evaluation if needed.**

Thanks for references

Yours

Prof. DR: Khaled Lakouz

Dr. Mohamed G. Menshawy, MSc



Ministry of Health and Family Welfare
Government of Punjab, Pakistan



MDT form

3-Medical numbers:
Female

63167451
Old Med

23/04/2023

A-patient ID: 28210011302441
1- Name: Ms. Nadeem Sex: Female

Patient Age: 42 Y



Family history	Age	Phone	01226283883	BMI	18.5	REASON OF MDT	23/04/2024
Marital status	Age	NRS score	10	GROUP	0		
Comorbidity	Age	Menopausal status	Postmenopausal	INITIAL STAGE	Stage II		

Clinical data

Breast Imaging: Metastatic
Metastatic
bone scan free

EX	ER	PR	Her 2 Neu	SISH	Ki67
IDC	2/8	4/8	3		15
T		N		M	
4d		2		0	

Sonomammography

PATIENT HISTORY
pt ended NACT 6 TC + 4 HP last 20/2/2024
on ex:nearly CR
It breast mastectomy+axillary LN dissection: no residual viable tumor
free resection margins, skin
complete pathological response

MDT OUTCOME & PLAN

complete 1 year phesgo,,, PORT,,, nolvadex 5 years

MEDICAL ONCOLOGY	SIGNATURE	RADIOLOGY	PATHOLOGY
SURGERY			
Psychiatric		Nutrition	

Presidential Initiative for Women's Health - Pathology Lab
المختبر المركزي لصحة المرأة - مستشفى الملكة فاطمة

Patient Name	: الاسم / غاده عبدالعزيز مصطفى	Request No.	: 13005622
Patient No.	: 2821001132441	Request Date	: 20/03/2024
Path No.	: 11111/24	Verification Date	: 27/03/2024
Sex	: Female	Financial Category	: فئة المرضى - أورام نادر النساء
Age	: 41 Y	Department	: Out Patient
Referring Doc	:	Registered by	: marwa mohamed youss

Pathology Report

- Resection Margins: Free.
- Skin: Free.
- Lymph nodes: Free axillary lymph nodes, L.N (0/17).
- Histologic response to therapy: Complete pathological response in previously diagnosed invasive duct carcinoma.
- Stage: ypT0, ypN0.

Signature

Prof. Dr. Iman Gouda.
Dr. Mohamed Turkey.

Verified by :
Dr.Duaa Mohamed

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Page 2 of 2

Printing By : marwa mohamed youss

CAP
ACCREDITED

غرفة رقم (4) - الأورام النادرة - مبنى الجراحات - مستشفى الملكة فاطمة - مصر الجديدة - القاهرة - مصر
Room No. (4) - First Minor-Building No. 987- Nile Corniche - Eltar El Qandana - Cairo, Egypt

Dr. M. Nabil El Bolkainy

Ph. D. (Path.) Michigan, U.S.A.

Dr. Tarek N. El Bolkainy

Ph. D. (Path.) Cairo University



دكتور محمد نبيل البلقيني

دكتوراه الباثولوجيا - جامعة ميشيغان

دكتور طارق نبيل البلقيني

دكتوراه الباثولوجيا - جامعة القاهرة

Name: علاء عبدالعزيز مصطفى السيد

Age: 41 Years Sex: Female

Clinical: Left breast highly suspicious mass with suspicious axillary lymph nodes

Referred By: Prof

ا.د. احمد توفيق

Date: 30-Aug-23

Gross

Multiple cores, totally submitted.

Microscopic

- * The breast tissue shows an invasive carcinoma, composed of nests and cords of cells with ductal differentiation. Tumor cells are large, pleomorphic, cohesive and show mitotic activity. There is stromal fibrosis of moderate degree. No associated major intraduct carcinoma. There is lymphovascular invasion.

Diagnosis

Breast, Left side, Biopsy: Invasive duct carcinoma, grade 2

Category

Dr. M. Nabil

{code: 7610 / 50 }

M. N. El Bolkainy, M.D. Ph. D.
Date: 31/08/2023

2 Sherif Street, Apt. 18 Cairo. Tel.: 23922022 - Fax : 23927964

٣٣٩٣٣٩١٤ : فاكس ٢٣٩٢٢٠٢٢ : القاهرة

٢ شارع شريف. عمارة اللؤلؤ. شقة ١٨. شارع شريف. عمارة اللؤلؤ. شقة ١٨. القاهرة. ت: ٢٣٩٢٢٠٢٢. فاكس: ٢٣٩٣٣٩١٤
لرجو الاحتفاظ بالشرائح المرفقة كسجل دائم للتشخيص / ويحتفظ بالعينات الجراحية لمدة أسبوع ولطوك الشمع الدال على المرض لمدة عام

Dr. M. Nabir El Bolkainy
Ph. D. (Path.) Michigan, U.S.A.

Dr. Tarek N. El Bolkainy
Ph. D. (Path.) Cairo University



دكتور محمد نبيل البلقيني
دكتوراه الباثولوجيا - جامعة ميشيغان
دكتور طارق نبيل البلقيني
دكتوراه الباثولوجيا - جامعة القاهرة

Name: غاده عبد العزيز مصطفى السيد

Age: 41 Years Sex: Female

Clinical: Paraffin blocks for determination of hormone receptors status.

Referred By: Prof احمد توفيق

Date: 07-Sep-23

Gross

Sections immunostained using supersensitive monoclonal antibodies with Dako Envision (Dextran substrate polymer) detection system. Microwave pretreatment for antigen retrieval and blocking of internal peroxidases were performed. DAB was utilized as a chromogen.

Microscopic

Tumor cells show no evidence of any nuclear staining for ER (-) or PR (-)

Diagnosis

ER and PR status: Negative (-)

Category

Hormone receptor immunohistochemical assay.

{code: 7931 / 0 }

M. N. El Bolkainy, M.D. Ph. D.
Date 10/09/2023

Dr. M. Nabil El Bolkainy
Ph. D. (Path.) Michigan, U.S.A.

Dr. Tarek N. El Bolkainy
Ph. D. (Path.) Cairo University



دكتور محمد نبيل البلقيني
دكتوراه الباثولوجيا - جامعة ميشيغان
دكتور طارق نبيل البلقيني
دكتوراه الباثولوجيا - جامعة القاهرة

Name: غاده عبد العزيز مصطفى السيد

Age: 41 Years Sex: Female

Clinical: Paraffin blocks for determination of C-erbB-2 (HER2/neu) status.

Referred By: Prof احمد توفيق

Date: 07-Sep-23

• Gross •

Sections immunostained for C-erbB-2 (HER2/neu) using supersensitive antibodies with Dako Envision (dextran substrate polymer) detection system. Microwave pretreatment for antigen retrieval and blocking of endogenous peroxidases were performed. DAB was used as a chromogen.

Microscopic

For the determination of HER2 protein overexpression, only the membrane staining intensity and pattern in the invasive part of tumor are evaluated. HER2 overexpression is interpreted as negative for both scores 0 (no staining is observed or membrane staining is observed in < 10% of tumor cells) and 1+ (faint incomplete membrane staining in > 10% of tumor cells). The test is interpreted as weakly positive (borderline or equivocal) for score 2+ (weak to moderate complete membrane staining in > 10% of tumor cells). A strong positive result is interpreted when the score is 3+ (strong complete membrane staining in > 10% of tumor cells).

• Diagnosis •

Immunostaining for C-erbB-2 (HER2/neu): Positive (Score 3+). Confirmatory gene expression profile is recommended and available on request. KI 67: 30%.

Category

Immunohistochemical assay.

{code: 7931 / 0 }

M. N. El Bolkainy, M.D. Ph. D.
Date 10/09/2023

2 Sherif Street, Apt. 18 Cairo. Tel.: 23922022 - Fax: 23927964

٢ شارع شريف، عمارة اللوك - شقة ١٨، القاهرة ت. ٢٣٩٢٢٠٢٢ فاكس: ٢٣٩٢٧٩٦٤

نرجو الاحتفاظ بالشرائح المرفقة كسجل دائم للتشخيص / ويحتفظ بالعينات الجراحية لمدة أسبوع ويطوك الشمع الدال على المرض لمدة عام



Radiotherapy Report

Patient Name: *Abdo jayda 2024*

Age: *21*
..... *SC*

Patient ID : *403/2024*

Diagnosis: *Lt Breast cancer*

Dose Prescription : *40 Gy (chest wall + Node)*

Technique: *3D CRT*

Number of Fractions: *15 f#*

Dose / fraction: *2.67 Gy/f*

Started : *7/5/2024* Finished : *26/5/2024*

Recommendations:

Hospital Manager



Doctor in charge



Presidential Initiative for Women's Health - Pathology Lab
المبادرة الرئاسية لصحة المرأة - معمل الباثولوجي

Patient Name	: الأستاذة / غادة عبدالجبار مصطفى	Request No.	: 13005622
Patient No.	: 2821001132441	Request Date	: 20/03/2024
Birth No.	: 1111/24	Verification Date	: 27/03/2024
Sex	: Female	Financial Category	: فئة المرأة - أورام دار السلام
Age	: 41 Y	Department	: Out Patient
Referring Doc :		Registered by	: marwa mohamed youss

Pathology Report

Clinical Data: Left breast cancer, post NACT.

Specimen: Left breast mastectomy & Axillary lymph nodes dissection.

Gross Examination:

Procedure: Left breast mastectomy measured 20x19x4 cm, with covering skin ellipse measured 17x11 cm.

Tumor extent: A greyish white rubbery to firm tumor bed site.

Size: 8 x7 x4 cm.

Skin: Nipple & areola looked unremarkable grossly.

Distance from margins: 0.3 cm away from deep margin.

Axilla: Fibrofatty tissue piece measured 5x5x3 cm, entangling 17 L.Ns, up to 2x1x0.5 cm.

Microscopic Examination:

Histological type: No residual viable tumor tissue.

Actual tumor cellularity: 0%.

Ductal carcinoma in situ: Not detected.

Skin: Free.

Resection Margins: Free.

Additional pathologic findings: Multiple fibroadenomas.

Lymph nodes: Free lymph nodes, L.N (0/17).

Histologic response to therapy: Complete pathological response to therapy in previously diagnosed invasive duct carcinoma.

Diagnosis:

Left breast cancer post NACT, Left breast mastectomy & Axillary lymph nodes dissection: No residual viable tumor tissue.

Actual tumor cellularity: 0%.

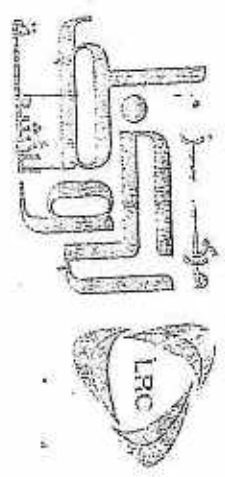
Verified by :

Dr.Duaa Mohamed

Printing Date : 03/04/2024 11:54:14

Page 1 of 2

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أ.أ. خالد أحمد لكؤز
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طبيب: عبد العزيز مصطفى	Study Date: 07/09/2023
PROF DR.: محمد عبد الجواد سليمان	Patient ID: 585214

MULTISLICE CT CHEST, ABDOMEN & PELVIS POST IV CONTRAST STUDY
REVEALS: Known case of LT. breast CA.

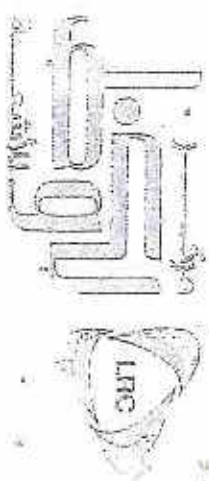
- Diffuse LT. breast skin thickening with dense glandular tissue and multiple LT. axillary metastatic LNs, ranging from 5-19 mm.
- Normal CT characteristics of both lungs with no focal pulmonary masses or infiltration.
- No mediastinal masses or LNs enlargement.
- No pleural or pericardial effusion.
- Average size, normal density liver with no focal masses or intrahepatic biliary dilatation.
- Normal size spleen with no focal masses.
- Both kidneys, GB & pancreas are displaying normal CT characteristics with no focal enhanced masses.
- No pelviabdominal LNs enlargement.
- Normal examined bowel loops with no focal masses.
- Normal UB.
- Normal size Uterus with an IUD in place. No adnexal masses.
- No osteolytic or sclerotic bony lesions.

CONCLUSION:

- o Unremarkable CT study.

Thanks for references
Yours
Dr Tamer Ibrahim
Prof. Dr: Khaled Lakouz

د. خالد محمد الجوعان
د. خالد محمد الجوعان
د. خالد محمد الجوعان



Patient Name: د. خالد محمد الجوعان	Study Date: 11-Sep-23
PROF DR.: د. خالد محمد الجوعان	Patient ID: 585214



History

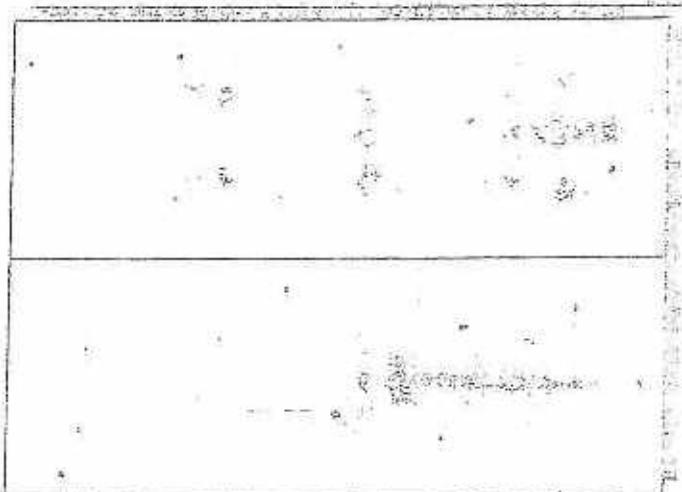
41 years old female patient presented with non-operated left cancer breast and referred for metastatic work-up.

Technique

Anterior and posterior whole body scans were acquired 2 hours post injection of 20 mCi of the tracer.

Findings

- The current study is free from focal osseous lesions that would account for bone metastases.
- Mild degenerative changes are noted at different levels of the spine.
- N.B. the increased tracer activity seen at the right sacroiliac region is likely of benign degenerative nature.



IMPRESSION :-

- The skeleton is free from metastases.

MUHAMMAD OMAR
Dr. AMIR M. SALAH, M.Sc.
PROF. DR. MAGUED ABDEL GALIL

أ.د. خالد أحمد لكويا
 أ.د. صبري رافع
 أ.د. ماهر عبدالجبار
 أ.د. كريم خالد لكويا
 أ.د. تامر محمد إبراهيم
 أ.د. محمد هشام صالح



Patient Name: <u>علاء عبدالعزيز مصطفى السيد</u>	Study Date: <u>26/08/2023</u>
PROF DR.: <u>أحمد توفيق</u>	Patient ID: <u>581860</u>

COMPUTED MAMMOGRAPHIC EXAMINATION OF BOTH BREASTS REVEAL:
CRANIO-CAUDAL & MEDIO-LATERAL OBLIQUE VIEWS WERE TAKEN.

- Normal parenchymal densities of both breasts with an ill-defined LT upper breast faint opaque breast mass harbors few calcifications with overlying skin thickening, & nipple retraction. No other focal breast masses.

US EXAMINATION OF BOTH BREASTS REVEAL:

- Normal echogenic pattern of both breasts with LT upper breast soft tissue mass (12 O'clock) measuring about 20x18 mm with lobulated borders, showing small foci of calcifications. It is associated with overlying skin thickening & nipple retraction.
- Multiple enlarged LT axillary LNs, the largest one measures about 22x16 mm most of them show lost central fatty hilum.
- Normal RT breast. No focal masses.

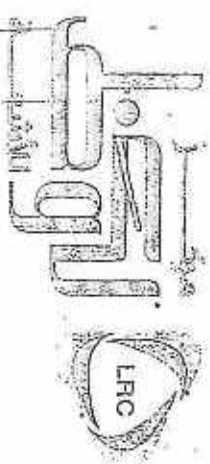
Conclusion:

- LT upper breast mass as described ... (BIRADS-IVc), For biops further evaluation if needed.

Thanks for references
 Yours

Prof. DR: Khaled Lakou
 Dr. Mohamed G. Menshawy, MS

د.د. خالد احمد الجوز
د.د. احمد صبري رافع
د.د. خالد الجليل
د.د. ناصر محمد الجرايدي
د.د. محمد هشام صالح



Patient Name: علاء عبدالرزاق مصطفى السيد Study Date: 07/09/2023
PROF DR.: محمد عبدالرحمن السليمان Patient ID: 585214

Echocardiography Report

2D AND M-MODE Measurements:

Ao root	26	(20-37) mm	LV EDD	46	(38-57) mm
LA	37	(20-40) mm	LV EDS	25	(22-40) mm
Ao exc.	20	(15-26) mm	E.F.	76%	50-75 %
RVD	27	(16-32) mm	F.S.	45%	25-45 %
IVST	9	(7-11) mm	MVA	Normal	4-6 cm ²
PWT	9	(7-11) mm	EPSS	-----	< 8 mm

*Comments:

- ◊ Normal left ventricular internal dimensions, normal echometry, normal wall thickness and normal LV functions.
- ◊ Normal LA dimension.
- ◊ LV Diastolic Dysfunction Grade I.
- ◊ Normal Cardiac valves flow and morphology.
- ◊ Normal pulmonary artery pressure (SPAP = 24 mmHg).
- ◊ Normal resting segmental wall motions.
- ◊ Intact intracardiac septae.
- ◊ No TTE evidence of intra-cardiac masses or thrombi.
- ◊ Free pericardium.

Conclusion:

LV Diastolic Dysfunction Grade I

Dr Ahmed Darwish, MD
Lecturer of cardiology

dr ahmed darwish

Date & Time: 07/09/2023 02:43:23

Patient Name: Ghada Abdel Aziz Moustafa Al Sayed	Date : 20/02/2024
Exam : ultrasound BREAST (BOTH SIDE)	Age : 41 Year(s)
Medical ID : 3856/2023	Pat. ID: 47061

DIGITAL BILATERAL SOFT TISSUE MAMMOGRAPHY AND COMPLEMENTARY ULTRASOUND

Indication:

PNCT assessment of left breast pathologically proven IDC.

Report:

- ACR c: heterogeneous fibroglandular parenchyma
 - Left Breast:
 - Irregular dense mass is seen in the UOQ with overlying skin thickening and microcalcification.
 - Ultrasound shows this mass as an irregular heterogeneous lobulated mass at 12 o'clock measuring about 21x12mm. mildly reduced in size with increased echogenicity denoting partial response.
 - No residual pathologically enlarged axillary lymph nodes.
 - No supra or infraclavicular lymph node.
 - Right Breast:
 - No suspicious micro calcifications or architectural distortion detected.
 - Complementary ultrasound no cystic or solid mass lesions.
 - Normal skin thickness, and nipple-areola complex with average caliber of the retro-areolar central ducts.
 - Nonspecific axillary LN.
- Opinion:**
- Post NACT status of left breast showing partial response of the previously noted malignant mass with no residual lymphadenopathy (BIRADS 6)
 - Normal sonomammography of right breast (BIRADS 1)
- Best regards,

D. Amal Ibrahim.
Dr. Amira H Radwan MD,
Breast Imaging consultant,
NCI, Cairo University
20/02/2024 3:01 PM

2021
J856-2023

2-10 number: 28210011302441

[illegible]

1000

23/04/2024 21

1997, 1998, 1999, 2000, 2001, 2002, 2003, 2004, 2005, 2006, 2007, 2008, 2009, 2010, 2011, 2012, 2013, 2014, 2015, 2016, 2017, 2018, 2019, 2020, 2021, 2022, 2023, 2024, 2025, 2026, 2027, 2028, 2029, 2030, 2031, 2032, 2033, 2034, 2035, 2036, 2037, 2038, 2039, 2040, 2041, 2042, 2043, 2044, 2045, 2046, 2047, 2048, 2049, 2050, 2051, 2052, 2053, 2054, 2055, 2056, 2057, 2058, 2059, 2060, 2061, 2062, 2063, 2064, 2065, 2066, 2067, 2068, 2069, 2070, 2071, 2072, 2073, 2074, 2075, 2076, 2077, 2078, 2079, 2080, 2081, 2082, 2083, 2084, 2085, 2086, 2087, 2088, 2089, 2090, 2091, 2092, 2093, 2094, 2095, 2096, 2097, 2098, 2099, 2100, 2101, 2102, 2103, 2104, 2105, 2106, 2107, 2108, 2109, 2110, 2111, 2112, 2113, 2114, 2115, 2116, 2117, 2118, 2119, 2120, 2121, 2122, 2123, 2124, 2125, 2126, 2127, 2128, 2129, 2130, 2131, 2132, 2133, 2134, 2135, 2136, 2137, 2138, 2139, 2140, 2141, 2142, 2143, 2144, 2145, 2146, 2147, 2148, 2149, 2150, 2151, 2152, 2153, 2154, 2155, 2156, 2157, 2158, 2159, 2160, 2161, 2162, 2163, 2164, 2165, 2166, 2167, 2168, 2169, 2170, 2171, 2172, 2173, 2174, 2175, 2176, 2177, 2178, 2179, 2180, 2181, 2182, 2183, 2184, 2185, 2186, 2187, 2188, 2189, 2190, 2191, 2192, 2193, 2194, 2195, 2196, 2197, 2198, 2199, 2200, 2201, 2202, 2203, 2204, 2205, 2206, 2207, 2208, 2209, 2210, 2211, 2212, 2213, 2214, 2215, 2216, 2217, 2218, 2219, 2220, 2221, 2222, 2223, 2224, 2225, 2226, 2227, 2228, 2229, 2230, 2231, 2232, 2233, 2234, 2235, 2236, 2237, 2238, 2239, 2240, 2241, 2242, 2243, 2244, 2245, 2246, 2247, 2248, 2249, 2250, 2251, 2252, 2253, 2254, 2255, 2256, 2257, 2258, 2259, 2260, 2261, 2262, 2263, 2264, 2265, 2266, 2267, 2268, 2269, 2270, 2271, 2272, 2273, 2274, 2275, 2276, 2277, 2278, 2279, 2280, 2281, 2282, 2283, 2284, 2285, 2286, 2287, 2288, 2289, 2290, 2291, 2292, 2293, 2294, 2295, 2296, 2297, 2298, 2299, 2300, 2301, 2302, 2303, 2304, 2305, 2306, 2307, 2308, 2309, 2310, 2311, 2312, 2313, 2314, 2315, 2316, 2317, 2318, 2319, 2320, 2321, 2322, 2323, 2324, 2325, 2326, 2327, 2328, 2329, 2330, 2331, 2332, 2333, 2334, 2335, 2336, 2337, 2338, 2339, 2340, 2341, 2342, 2343, 2344, 2345, 2346, 2347, 2348, 2349, 2350, 2351, 2352, 2353, 2354, 2355, 2356, 2357, 2358, 2359, 2360, 2361, 2362, 2363, 2364, 2365, 2366, 2367, 2368, 2369, 2370, 2371, 2372, 2373, 2374, 2375, 2376, 2377, 2378, 2379, 2380, 2381, 2382, 2383, 2384, 2385, 2386, 2387, 2388, 2389, 2390, 2391, 2392, 2393, 2394, 2395, 2396, 2397, 2398, 2399, 2400, 2401, 2402, 2403, 2404, 2405, 2406, 2407, 2408, 2409, 2410, 2411, 2412, 2413, 2414, 2415, 2416, 2417, 2418, 2419, 2420, 2421, 2422, 2423, 2424, 2425, 2426, 2427, 2428, 2429, 2430, 2431, 2432, 2433, 2434, 2435, 2436, 2437, 2438, 2439, 2440, 2441, 2442, 2443, 2444, 2445, 2446, 2447, 2448, 2449, 2450, 2451, 2452, 2453, 2454, 2455, 2456, 2457, 2458, 2459, 2460, 2461, 2462, 2463, 2464, 2465, 2466, 2467, 2468, 2469, 2470, 2471, 2472, 2473, 2474, 2475, 2476, 2477, 2478, 2479, 2480, 2481, 2482, 2483, 2484, 2485, 2486, 2487, 2488, 2489, 2490, 2491, 2492, 2493, 2494, 2495, 2496, 2497, 2498, 2499, 2500, 2501, 2502, 2503, 2504, 2505, 2506, 2507, 2508, 2509, 2510, 2511, 2512, 2513, 2514, 2515, 2516, 2517, 2518, 2519, 2520, 2521, 2522, 2523, 2524, 2525, 2526, 2527, 2528, 2529, 2530, 2531, 2532, 2533, 2534, 2535, 2536, 2537, 2538, 2539, 2540, 2541, 2542, 2543, 2544, 2545, 2546, 2547, 2548, 2549, 2550, 2551, 2552, 2553, 2554, 2555, 2556, 2557, 2558, 2559, 2560, 2561, 2562, 2563, 2564, 2565, 2566, 2567, 2568, 2569, 2570, 2571, 2572, 2573, 2574, 2575, 2576, 2577, 2578, 2579, 2580, 2581, 2582, 2583, 2584, 2585, 2586, 2587, 2588, 2589, 2590, 2591, 2592, 2593, 2594, 2595, 2596, 2597, 2598, 2599, 2600, 2601, 2602, 2603, 2604, 2605, 2606, 2607, 2608, 2609, 2610, 2611, 2612, 2613, 2614, 2615, 2616, 2617, 2618, 2619, 2620, 2621, 2622, 2623, 2624, 2625, 2626, 2627, 2628, 2629, 2630, 2631, 2632, 2633, 2634, 2635, 2636, 2637, 2638, 2639, 2640, 2641, 2642, 2643, 2644, 2645, 2646, 2647, 2648, 2649, 2650, 2651, 2652, 2653, 2654, 2655, 2656, 2657, 2658, 2659, 2660, 2661, 2662, 2663, 2664, 2665, 2666, 2667, 2668, 2669, 2670, 2671, 2672, 2673, 2674, 2675, 2676, 2677, 2678, 26

et cap: no metals
bone scan: free

Psychiatric

Presidential Initiative for Women's Health - Pathology Lab
الخدمات الطبية النسائية - مستشفى الملكة فاطمة

Patient Name	: الاسم / غادة عبدالعزیز مصطفى	Request No.	: 13005622
Patient No.	: 2821001132441	Request Date	: 20/03/2024
Path No.	: 11111/24	Verification Date	: 27/03/2024
Sex	: Female	Financial Category	: فئة المرضى - أورام نادر النساء
Age	: 41 Y	Department	: Out Patient
Referring Doc	:	Registered by	: marwa mohamed youss

Pathology Report

- Resection Margins: Free.
- Skin: Free.
- Lymph nodes: Free axillary lymph nodes, L.N (0/17).
- Histologic response to therapy: Complete pathological response in previously diagnosed invasive duct carcinoma.
- Stage: ypT0, ypN0.

Signature

Prof. Dr. Iman Gouda.
Dr. Mohamed Turkey.

Verified by :
Dr.Duaa Mohamed

Printing Date : 03/04/2024 11:54:14

Page 2 of 2

Printing By : marwa mohamed youss

CAP
ACCREDITED

غرفة رقم (4) - الأورام نادر النساء - (987) - مستشفى الملكة فاطمة - مصر الجديدة - القاهرة - مصر
Room No. (4) - First Minor-Building No. 987- Nile Corniche - Eltar El Gendana - Cairo, Egypt

Dr. M. Nabil El Bolkainy

Ph. D. (Path.) Michigan, U.S.A.

Dr. Tarek N. El Bolkainy

Ph. D. (Path.) Cairo University



دكتور محمد نبيل البلقيني

دكتوراه الباثولوجيا - جامعة ميشيغان

دكتور طارق نبيل البلقيني

دكتوراه الباثولوجيا - جامعة القاهرة

Name: علاء عبدالعزيز مصطفى السيد

Age: 41 Years Sex: Female

Clinical: Left breast highly suspicious mass with suspicious axillary lymph nodes

Referred By: Prof

ا.د. احمد توفيق

Date: 30-Aug-23

Gross

Multiple cores, totally submitted.

Microscopic

- * The breast tissue shows an invasive carcinoma, composed of nests and cords of cells with ductal differentiation. Tumor cells are large, pleomorphic, cohesive and show mitotic activity. There is stromal fibrosis of moderate degree. No associated major intraduct carcinoma. There is lymphovascular invasion.

Diagnosis

Breast, Left side, Biopsy: Invasive duct carcinoma, grade 2

Category

Dr. M. Nabil

{code: 7610 / 50 }

M. N. El Bolkainy, M.D. Ph. D.
Date: 31/08/2023

2 Sherif Street, Apt. 18 Cairo. Tel.: 23922022 - Fax : 23927964

٣٣٩٣٣٩١٤ : فاكس ٢٣٩٢٢٠٢٢ : القاهرة

٢ شارع شريف. عمارة اللؤلؤ - شقة ١٨. شارع شريف. جامعة القاهرة
لرجو الاحتفاظ بالشرائح المرفقة كسجل دائم للتشخيص / ويحتفظ بالعينات الجراحية لمدة أسبوع ولطوك الشمع الدال على المرض لمدة عام

Dr. M. Nabif El Bolkainy

Ph. D. (Path.) Michigan, U.S.A.

Dr. Tarek N. El Bolkainy

Ph. D. (Path.) Cairo University



دكتور محمد نبيل البلقيني

دكتوراه الباثولوجيا - جامعة ميشيغان

دكتور طارق نبيل البلقيني

دكتوراه الباثولوجيا - جامعة القاهرة

Name: غاده عبد العزيز مصطفى السيد

Age: 41 Years Sex: Female

Clinical: Paraffin blocks for determination of hormone receptors status.

Referred By: Prof احمد توفيق

Date: 07-Sep-23

Gross

Sections immunostained using supersensitive monoclonal antibodies with Dako Envision (Dextran substrate polymer) detection system. Microwave pretreatment for antigen retrieval and blocking of internal peroxidases were performed. DAB was utilized as a chromogen.

Microscopic

Tumor cells show no evidence of any nuclear staining for ER (-) or PR (-)

Diagnosis

ER and PR status: Negative (-)

Category

Hormone receptor immunohistochemical assay.

{code: 7931 / 0 }

M. N. El Bolkainy, M.D. Ph. D.
Date 10/09/2023

2 Sherif Street, Apt. 18 Cairo. Tel.: 23922022 - Fax: 23927964

شارع شريف، عمارة اللؤلؤ، شقة ١٨، القاهرة. ت: ٢٣٩٢٢٠٢٢، فاكس: ٢٣٩٢٧٩٦٤
نرجو الاحتفاظ بالشرائح المرفقة كسجل دائم للتشخيص، ويحتفظ بالعينات الجراحية لمدة أسبوع وولوج الشمع الدال على المرض لمدة عام

Dr. M. Nabil El Bolkainy
Ph. D. (Path.) Michigan, U.S.A.

Dr. Tarek N. El Bolkainy
Ph. D. (Path.) Cairo University



دكتور محمد نبيل البلقيني
دكتوراه الباثولوجيا - جامعة ميشيغان
دكتور طارق نبيل البلقيني
دكتوراه الباثولوجيا - جامعة القاهرة

Name: غاده عبد العزيز مصطفى السيد

Age: 41 Years Sex: Female

Clinical: Paraffin blocks for determination of C-erbB-2 (HER2/neu) status.

Referred By: Prof احمد توفيق

Date: 07-Sep-23

• Gross •

Sections immunostained for C-erbB-2 (HER2/neu) using supersensitive antibodies with Dako Envision (dextran substrate polymer) detection system. Microwave pretreatment for antigen retrieval and blocking of endogenous peroxidases were performed. DAB was used as a chromogen.

Microscopic

For the determination of HER2 protein overexpression, only the membrane staining intensity and pattern in the invasive part of tumor are evaluated. HER2 overexpression is interpreted as negative for both scores 0 (no staining is observed or membrane staining is observed in < 10% of tumor cells) and 1+ (faint incomplete membrane staining in > 10% of tumor cells). The test is interpreted as weakly positive (borderline or equivocal) for score 2+ (weak to moderate complete membrane staining in > 10% of tumor cells). A strong positive result is interpreted when the score is 3+ (strong complete membrane staining in > 10% of tumor cells).

• Diagnosis •

Immunostaining for C-erbB-2 (HER2/neu): Positive (Score 3+). Confirmatory gene expression profile is recommended and available on request. KI 67: 30%.

Category

Immunohistochemical assay.

{code: 7931 / 0 }

M. N. El Bolkainy, M.D. Ph. D.
Date 10/09/2023

2 Sherif Street, Apt. 18 Cairo. Tel.: 23922022 - Fax: 23927964

٢ شارع شريف، عمارة اللوك - شقة ١٨، القاهرة. ت: ٢٣٩٢٢٠٢٢ - فاكس: ٢٣٩٢٧٩٦٤

نرجو الاحتفاظ بالشرائح المرفقة كسجل دائم للتشخيص / ويحتفظ بالعينات الجراحية لمدة أسبوع ويطوك الشمع الدال على المرض لمدة عام



Radiotherapy Report

Patient Name: *Abdo jayda 2015*

Age: *21 y/c*

Patient ID : *403/2024*

Diagnosis: *Lt Breast cancer*

Dose Prescription : *40 Gy (chest wall + Node)*

Technique:..... *3D CRT*

Number of Fractions: *15 f#*

Dose / fraction: *2.67 Gy/f*

Started : *7/5/2024* Finished : *26/5/2024*

Recommendations:.....

.....

Hospital Manager



Doctor in charge



Presidential Initiative for Women's Health - Pathology Lab
المبادرة الرئاسية لصحة المرأة - معمل الباثولوجي

Patient Name	: الأستاذة / غادة عبدالجبار مصطفى	Request No.	: 13005622
Patient No.	: 2821001132441	Request Date	: 20/03/2024
Birth No.	: 1111/24	Verification Date	: 27/03/2024
Sex	: Female	Financial Category	: فئة المرأة - أورام دار السلام
Age	: 41 Y	Department	: Out Patient
Referring Doc	:	Registered by	: marwa mohamed youss

Pathology Report

Clinical Data: Left breast cancer, post NACT.

Specimen: Left breast mastectomy & Axillary lymph nodes dissection.

Gross Examination:

Procedure: Left breast mastectomy measured 20x19x4 cm, with covering skin ellipse measured 17x11 cm.
Tumor extent: A greyish white rubbery to firm tumor bed site.
Size: 8 x7 x4 cm.
Skin: Nipple & areola looked unremarkable grossly.
Distance from margins: 0.3 cm away from deep margin.
Axilla: Fibrofatty tissue piece measured 5x5x3 cm, entangling 17 L.Ns, up to 2x1x0.5 cm.

Microscopic Examination:

Histological type: No residual viable tumor tissue.
Actual tumor cellularity: 0%.
Ductal carcinoma in situ: Not detected.
Skin: Free.
Resection Margins: Free.
Additional pathologic findings: Multiple fibroadenomas.
Lymph nodes: Free lymph nodes, L.N (0/17).
Histologic response to therapy: Complete pathological response to therapy in previously diagnosed invasive duct carcinoma.

Diagnosis:

Left breast cancer post NACT, Left breast mastectomy & Axillary lymph nodes dissection: No residual viable tumor tissue.
Actual tumor cellularity: 0%.

Verified by :

Dr.Duaa Mohamed

Printing Date : 03/04/2024 11:54:14

Page 1 of 2

Printing By : marwa mohamed youss

Pt Name : عبد الله بن محمد بن عبد الله / Abdullah M	Request No. : 13004450
Ref No. : 28210011302441	Request Date : 19/10/2023
Io. : SR-1234/23	Verification Date : 02/11/2023
: Female	Financial Category : مريض - مريض
: 41 Y	Department : Out Patient
Refng Doc. : 	IHC No. :
	Registered by : marwa mohamed youssef

IMMUNOHISTOCHEMISTRY REPORT

11 Data: Left breast mass.
2 of Specimen: Slide revision, IPT (ER,PR,Her2)REPEATED IN OUR LAB

Technique:
Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana)
and by rabbit monoclonal antibodies using Ultraview HRP DAB as chromogen and hematoxylin as
counter-stain. The sections were treated against ER clone (1E2), PR clone (SP1), Her2 clone (4B5) and
Ki67 clone (30-9).

Interpretation:

- 1) Positive control showed appropriate reactivity.
- 2) Weak nuclear reaction in 1% of tumor cells (Allred score 2/8).
- 3) Weak nuclear reaction in 30% tumor cells (Allred score 4/8).
- 4) Strong complete membrane reaction in >10% tumor cells (score +3).
- 5) 15% nuclear reaction of tumor cells.

Diagnosis:
Invasive duct carcinoma, Grade II,
repeated in our lab

- 1) Negative (score 2/8).
- 2) Positive (score 4/8).
- 3) Positive (score +3).
- 4) Positive in 15% of tumor cells.

Signature

Verified by :

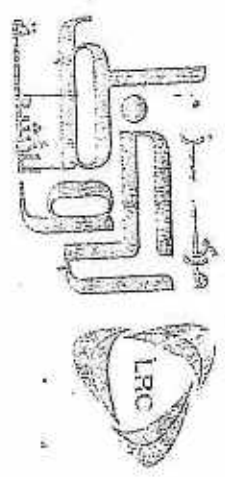
Dr.Heba Hasan

Printing Date : 02/11/2023 11:46:42 Page 1 of 2

Printed By : marwa mohamed youssef

CAP

مركز - القاهرة - الجيزة - مصر
Room No. (4) - First Floor-Building No. 987, Nile Fortification - El-Dokki - Cairo, Egypt



أ.أ. خالد أحمد الحكوذا
أ.أ. أحمد صبري رافع
أ.أ. خالد الحكوذا
أ.أ. خالد الحكوذا
أ.أ. خالد الحكوذا
أ.أ. خالد الحكوذا
أ.أ. خالد الحكوذا
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أ.أ. خالد الحكوذا
أ.أ. خالد الحكوذا

Patient Name: طاهر عبدالعزیز مصطفی العبد	Study Date: 07/09/2023
PROF DR.: محمد عبدالجواد ساهبان	Patient ID: 585214

MULTISLICE CT CHEST, ABDOMEN & PELVIS POST IV CONTRAST STUDY
REVEALS: Known case of LT. breast CA.

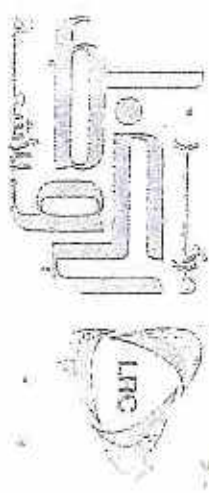
- Diffuse LT. breast skin thickening with dense glandular tissue and multiple LT. axillary metastatic LNs, ranging from 5-19 mm.
- Normal CT characteristics of both lungs with no focal pulmonary masses or infiltration.
- No mediastinal masses or LNs enlargement.
- No pleural or pericardial effusion.
- Average size, normal density liver with no focal masses or intrahepatic biliary dilatation.
- Normal size spleen with no focal masses.
- Both kidneys, GB & pancreas are displaying normal CT characteristics with no focal enhanced masses.
- No pelviabdominal LNs enlargement.
- Normal examined bowel loops with no focal masses.
- Normal UB.
- Normal size Uterus with an IUD in place. No adnexal masses.
- No osteolytic or sclerotic bony lesions.

CONCLUSION:

- o Unremarkable CT study.

Thanks for references
Yours
Dr Tamer Ibrahim
Prof. Dr: Khaled Lakouz

د. خالد محمد الجوعان
د. خالد محمد الجوعان
د. خالد محمد الجوعان



Patient Name: د. خالد محمد الجوعان	Study Date: 11-Sep-23
PROF DR.: د. خالد محمد الجوعان	Patient ID: 585214

History

41 years old female patient presented with non-operated left cancer breast and referred for metastatic work-up.

Technique

Anterior and posterior whole body scans were acquired 2 hours post injection of 20 mCi of the tracer.

Findings

- The current study is free from focal osseous lesions that would account for bone metastases.
- Mild degenerative changes are noted at different levels of the spine.
- N.B. the increased tracer activity seen at the right sacroiliac region is likely of benign degenerative nature.

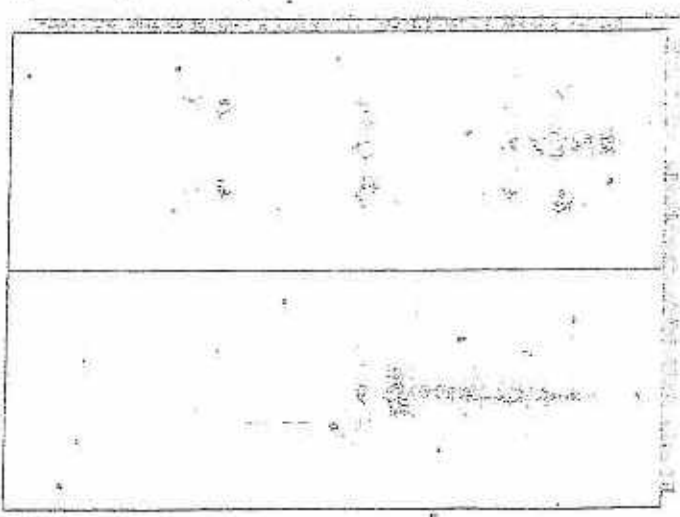
IMPRESSION :-

- The skeleton is free from metastases.

MUHAMMAD OMAR

Dr. AMIR M. SALAH, M.Sc.

PROF. DR. MAGUED ABDEL GALIL



أ.د. خالد أحمد لكويا
 أ.د. أحمد صبري رافع
 أ.د. ماهر عبدالجبار
 أ.د. كريم خالد لكويا
 أ.د. محمد هشام صالح



Patient Name: <u>علاء عبدالعزيز مصطفى السيد</u>	Study Date: <u>26/08/2023</u>
PROF DR.: <u>أحمد توفيق</u>	Patient ID: <u>581860</u>

COMPUTED MAMMOGRAPHIC EXAMINATION OF BOTH BREASTS REVEAL:
CRANIO-CAUDAL & MEDIO-LATERAL OBLIQUE VIEWS WERE TAKEN.

-Normal parenchymal densities of both breasts with an ill-defined LT upper breast faint opaque breast mass harbors few calcifications with overlying skin thickening, & nipple retraction. No other focal breast masses.

US EXAMINATION OF BOTH BREASTS REVEAL:

-Normal echogenic pattern of both breasts with LT upper breast soft tissue mass (12 O'clock) measuring about 20x18 mm with lobulated borders, showing small foci of calcifications. It is associated with overlying skin thickening & nipple retraction.
 -Multiple enlarged LT axillary LNs, the largest one measures about 22x16 mm most of them show lost central fatty hilum.
 -Normal RT breast. No focal masses.

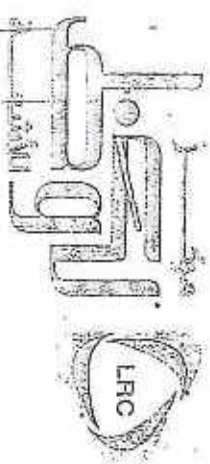
Conclusion:
 ... (BIRADS-IVc), For biops

• LT upper breast mass as described ... (BIRADS-IVc), For biops further evaluation if needed.

Thanks for references
 Yours

Prof. DR: Khaled Lakou
 Dr. Mohamed G. Menshawy, MS

د.د. خالد احمد الجوز
د.د. احمد صبري رافع
د.د. خالد الجليل
د.د. ناصر محمد الجرايدي
د.د. محمد هاشم صالح



Patient Name: علاء عبدالرزاق مصطفى السيد	Study Date: 07/09/2023
PROF DR.: محمد عبدالرزاق سليمان	Patient ID: 585214

Echocardiography Report

2D AND M-MODE Measurements:

Ao root	26	(20-37) mm	LV EDD	46	(38-57) mm
LA	37	(20-40) mm	LV EDS	25	(22-40) mm
Ao exc.	20	(15-26) mm	E.F.	76%	50-75 %
RVD	27	(16-32) mm	F.S.	45%	25-45 %
IVST	9	(7-11) mm	MVA	Normal	4-6 cm ²
PWT	9	(7-11) mm	EPSS	-----	< 8 mm

*Comments:

- ◊ Normal left ventricular internal dimensions, normal echometry, normal wall thickness and normal LV functions.
- ◊ Normal LA dimension.
- ◊ LV Diastolic Dysfunction Grade I.
- ◊ Normal Cardiac valves flow and morphology.
- ◊ Normal pulmonary artery pressure (SPAP = 24 mmHg).
- ◊ Normal resting segmental wall motions.
- ◊ Intact intracardiac septae.
- ◊ No TTE evidence of intra-cardiac masses or thrombi.
- ◊ Free pericardium.

Conclusion:

LV Diastolic Dysfunction Grade I

Dr Ahmed Darwish, MD
Lecturer of cardiology

dr ahmed darwish

Date & Time: 07/09/2023 02:43:23

Patient Name: Ghada Abdel Aziz Moustafa Al Sayed	Date : 20/02/2024
Exam : ultrasound BREAST (BOTH SIDE)	Age : 41 Year(s)
Medical ID : 3856/2023	Pat. ID: 47061

DIGITAL BILATERAL SOFT TISSUE MAMMOGRAPHY AND COMPLEMENTARY ULTRASOUND

Indication:

PNCT assessment of left breast pathologically proven IDC.

Report:

- ACR c: heterogeneous fibroglandular parenchyma
 - Left Breast:
 - Irregular dense mass is seen in the UOQ with overlying skin thickening and microcalcification.
 - Ultrasound shows this mass as an irregular heterogeneous lobulated mass at 12 o'clock measuring about 21x12mm. mildly reduced in size with increased echogenicity denoting partial response.
 - No residual pathologically enlarged axillary lymph nodes.
 - No supra or infraclavicular lymph node.
 - Right Breast:
 - No suspicious micro calcifications or architectural distortion detected.
 - Complementary ultrasound no cystic or solid mass lesions.
 - Normal skin thickness, and nipple-areola complex with average caliber of the retro-areolar central ducts.
 - Nonspecific axillary LN.
- Opinion:**
- Post NACT status of left breast showing partial response of the previously noted malignant mass with no residual lymphadenopathy (BIRADS 6)
 - Normal sonomammography of right breast (BIRADS 1)
- Best regards,

D. Amal Ibrahim.
Dr. Amira H Radwan MD,
Breast Imaging consultant,
NCI, Cairo University
20/02/2024 3:01 PM